

Setting up and running Lyfe Place Abuja

The mandate, the leanest way to fund it, and what Consult for Africa charges.

SUMMARY

Open it for NGN 307M. Earn NGN 547M a year from it.

The same building, let room by room to consultants recruited one at a time, returns about NGN 273M a year and takes two and a half years to repay. Configured as a medipark, with Medbury's own clinical businesses moved into it and run as one platform, it returns **NGN 547M a year**. And it can be opened for a fraction of what a conventional build would cost, because almost everything that can be leased, placed, consigned or deferred should be.

	NGN M	Note
Cash to open the campus	307	Leased, deferred or carried by the business that owns it
CFA mobilisation and entity kickoff	71	The rest of CFA's setup fee is paid from trading, or from the capital it saves
Cash before trading	354	Against NGN 662M on a conventional build
Rent and fees already paid	115	Two years in advance, already committed
Total exposure before a naira of revenue	469	Against NGN 897M the conventional way
Returns, before the management fee	547	A year, at stabilisation
Capital back	Year 2	Against year 3 letting the same rooms by the session

The principle running through this proposal is that capital should be spent last, not first. Solar comes on a power purchase agreement rather than a purchase. The laboratory and the pharmacy are equipped and stocked by the Medbury businesses that own them, on their own budgets. The aesthetics unit opens injectables-led, which needs almost no equipment, and buys no laser until the volume justifies leasing one. The theatre waits for a signed surgical anchor. **CFA applies the same rule to its own fee: most of it is paid from trading rather than before it.**

01 / THE MANDATE

What is actually being outsourced.

This is not an advisory engagement with a delivery component. It is a build-and-operate mandate over an asset Medbury owns and has already begun funding. The distinction matters for pricing, because an adviser is paid for judgement and an operator is paid for outcome.

The job	What it means here	Priced as
Set it up	Design, fit-out, power, equipping, regulatory approvals, commissioning and handover of a 711 sqm four-structure campus, on the leanest capital that will carry it	Delivery
Stand the businesses up	Structure, incorporate, licence, credential, write protocols, recruit and launch each anchor clinical entity	Establishment
Run the facility	Front office, bookings, billing and collection, patient navigation, facilities, compliance, clinical governance and reporting	Management
Run the businesses	Carry the P&L of the anchor clinical entities: pricing, throughput, clinical staffing, procurement, margin	Management
Maximise the return	The membership address, the family practice, the sessional letting business, the diagnostics and pharmacy attach	Management

Four anchor clinical entities, plus the initiatives. The anchors are the aesthetics and regenerative unit, the longevity and infusion unit, hair restoration, and the Alameda conversion clinic. The initiatives are the specialist membership address, the family practice, the sessional letting of the remaining rooms, and the diagnostics and pharmacy capture. **The anchors pay for the building. The initiatives are what make the return exceptional rather than adequate**, and they are the part that does not happen unless somebody is contracted to make it happen.

02 / WHAT THE MANDATE CREATES

The difference between two ways of using one building.

	Let by the session	As a medipark	Note
Rooms let by the session	5	3	Three is enough once businesses occupy the rest
Businesses in occupation	nil	6	Each with its own P&L, staff and patients
Specialists to recruit before it works	60	25	The businesses arrive already formed
Time to stabilisation	24 months	12 to 15 months	The ramp, not the rate card, decides when the money comes back
Campus-wide revenue	517	1,697	NGN M a year
To Medbury before any fee	253	547	NGN M a year, on the lean capital structure
Cash to open	647	307	NGN M

NGN 321M a year of value that does not exist unless somebody builds it and runs it, on barely a quarter of the capital. That is what this fee is a share of. A management fee benchmarked against what a facility manager costs to employ would be measuring the wrong thing: the campus does not need supervising, it needs to be brought into existence, filled with businesses that do not yet trade, and run as one platform. **CFA takes no share at all of the NGN 273M Medbury would have earned without it.**

Where the NGN 1,697M comes from

Business	Where it sits	Revenue	EBITDA	Medbury
Medlyfe Aesthetics and Regenerative	2 rooms and theatre access	475	97	51%
Medlyfe Longevity and Infusion	1 room, four infusion bays	240	71	100%
Lyfe Place Hair Restoration	The FUE suite	225	56	100%
Medbury Diagnostics	Laboratory, X-ray, ultrasound, echo	145	57	100%
Sessional panel and membership	3 rooms	142	n/a	100%
Alameda Conversion Clinic SPV	2 ground floor rooms	132	25	49%
Medbury Family Practice	1 room	128	25	100%
Medbury Pharmaceuticals	Boys' quarters and collection point	120	20	100%
Theatre lists, external	The day-case theatre	90	n/a	100%
Campus-wide		1,697		

Each business is set out in full, with its service menu, capacity, staffing, price points and cost structure, in the companion medipark business plan.

03 / THE SETUP COST

Every line, at the leanest specification that works.

A conventional build of this scope costs about NGN 750M before a patient walks in. Almost none of that is necessary at the start. The rule applied to every line is simple: **if it can be leased, placed by a supplier, taken on consignment, paid monthly, or deferred until the revenue that justifies it exists, it is not bought with cash on day one.** What follows is the whole schedule at that specification. Nothing is rolled into a round number.

	NGN M	Nature
A Property and statutory		
Change of use approval and development control	3.5	One-off
Facility registration, FCT health authority	2.0	One-off
Fire safety certificate	1.0	One-off
Environmental and clinical waste permit	0.8	One-off
Signage permit	0.4	One-off
B Building works		
Partitioning to form the consulting rooms	8.0	Capital
Joinery: reception desk, nurse stations, room casework	8.0	Capital
Painting and decoration	7.0	Capital
Enabling: strip out, minor structural, roof repairs	6.0	Capital
External: car park, lighting, drainage, gate house	6.0	Capital
Sanitary and plumbing, clinical hand-wash, condensate	5.0	Capital
Ceilings	4.0	Capital
Clinical flooring, welded and coved vinyl, clinical areas only	4.0	Capital
Doors and ironmongery	3.0	Capital
C Building services		
Cooling, split units and cassettes	16.0	Capital
ICT: cabling, network, CCTV, access control, emergency call	11.0	Capital
Electrical: boards, zoning, medical circuits, emergency lighting	10.0	Capital
Fire detection, alarm and escape lighting	8.0	Capital
Mechanical fresh air and extract	5.0	Capital
Power: changeover and distribution only, array on a PPA	3.0	Capital
Water: treatment, storage, pressure, hot water	3.0	Capital
Medical waste holding and route	3.0	Capital
D Furniture and fittings, non-clinical		
Consulting, treatment and infusion room furniture	6.0	Capital
Reception, waiting and concierge	5.0	Capital
Signage and wayfinding	4.0	Capital
Doctors' and members' lounge	3.0	Capital
Back of house, records, staff room	2.0	Capital
E Clinical equipment		
Bought at launch, scheduled item by item in section 04	45.5	Capital
F Technology and systems		
Workstations, printers, card readers, tablets	6.0	Capital
Practice management and billing, implementation, on subscription	2.5	One-off
Website and booking page	1.5	One-off
Telephony and internet installation	1.5	Capital
G Professional and regulatory		
Legal: four incorporations, shareholder agreements, leases, supplier contracts	8.0	One-off

	NGN M	Nature
Architectural and interior design	5.0	One-off
M&E consultant	3.0	One-off
Clinician credentialing, MDCN and NMCN, practice licences	3.0	One-off
Quantity surveyor, the independent budget benchmark	2.0	One-off
Insurance, deposit and first instalment, paid monthly thereafter	2.0	One-off
H People, before opening		
Payroll before revenue, core team phased against opening dates	10.0	One-off
Training and induction	4.0	One-off
Recruitment and selection costs	3.0	One-off
I Marketing and launch		
Pre-opening campaign, three months	9.0	One-off
Brand, identity, collateral, clinical photography	5.0	One-off
Launch event and clinician outreach	4.0	One-off
J Opening inventory		
Aesthetic product and injectables	4.0	Capital
Clinical consumables, campus	3.0	Capital
Non-clinical: linen, uniforms, stationery, housekeeping	2.0	Capital
K Working capital		
Operating float to cash-positive	45.0	Working capital
Cash to open	306.7	
	NGN M	Note
Capital, assets that last	192.0	Building, services, fittings, equipment, hardware
One-off setup cost	69.7	Statutory, professional, people, marketing. Expensed, not capitalised
Working capital	45.0	The float to cash-positive
Cash to open	306.7	
Rent and fees already paid	115.0	Two years in advance, already committed
CFA mobilisation and entity kickoff	47.0	The rest of CFA's setup fee is paid from trading, or from the capital it saves
Total before trading	468.7	Against about NGN 900M the conventional way

This number is NGN 102M higher than the round figure carried in earlier drafts, and the difference is not a change of plan. It is the statutory approvals, the professional fees, the people employed before there is revenue to pay them, and the marketing that has to run before the doors open. None of it is optional, none of it was in the original NGN 662M either, and all of it is cash Medbury would have had to find. **A setup budget that omits these is not lean, it is incomplete,** and the difference always appears later as an overrun.

Where the leanness actually comes from

Line	Convention al	Lean	How the balance is carried
Theatre suite	120	nil	Deferred until a surgical anchor is signed, then leased. The least evidenced demand on the campus and the largest single line
Hybrid solar array	33	3	Power purchase agreement with a lease-to-own tail. A monthly tariff against the diesel and grid spend it replaces, title transfers in years five to seven. Only the changeover and distribution is bought
Laser, IPL and body contouring	60	nil	Leased from month nine and deferred respectively. Injectables carry the aesthetics unit and need neither
Laboratory and pharmacy	52	nil	Equipped and stocked by Medbury Diagnostics and Medbury Pharmaceuticals on their own budgets
FUE instruments	14	nil	Brought by the hair restoration operator under a revenue share
Ground floor completion	75	nil	Phase two, out of trading
Insurance, systems, opening stock	24	9	Monthly instalments, subscription rather than licence purchase, and supplier terms
Taken out of day one	378	12	

What leaning costs, stated honestly

	NGN M a year
Equipment lease cost above the amortisation it replaces	28.0
Laboratory and pharmacy financing, decided by those businesses	17.5
Solar tariff above the cost of owning the array	6.0
Annual cost of leaning	51.5

NGN 51.5M a year of margin given up to avoid NGN 445M of capital. It would take nearly nine years of that margin to repay what is not being spent, and Medbury keeps the cash in the meantime. **The amount at risk if the campus disappoints falls from about NGN 900M to NGN 469M.** This is not a cheaper campus. It is the same campus, financed so that the building proves itself before the balance sheet commits to it. **Every lease and placement carries a buy-out option priced at signature and exercisable from year two**, so the structure is a financing choice Medbury can reverse out of trading cash rather than a permanent premium.

Phase two, funded from trading and not from a second cheque

	NGN M	When
Ground floor completion and the balance of the fit-out	75	Once the first floor is trading
Theatre suite, leased, deposit only	35	Once a surgical anchor is signed
Phase two	110	Out of cash flow. No further capital call

Three things should be tested during design and could reduce the cash further: a landlord contribution or rent-free period against a change of use that improves the property, vendor finance on the fit-out itself, and whether the hair restoration operator will bring their own instruments in exchange for a larger revenue share.

04 / THE EQUIPMENT

Scheduled, not estimated.

Equipment is the line most often waved through at a round number and then found to be twice it. This is the schedule. Four rules decide how each item is acquired: **buy what is cheap and essential, lease what is expensive and productive, defer what depends on demand that is not yet evidenced, and let a partner bring what only that partner will use.**

Item	Qty	NGN M	Route
Aesthetics and Regenerative			
Electric treatment couch	2	2.4	Buy
Procedure trolleys, mayo stands, stools	1	0.8	Buy
Pharmaceutical fridge with data logger	1	1.2	Buy
Class B benchtop autoclave	1	2.2	Buy
Minor procedure instrument sets	3	1.5	Buy
Magnifying lamp and dermatoscope	1	0.6	Buy
PRP centrifuge	1	1.5	Buy
Standardised clinical photography station	1	1.2	Buy
Sharps and clinical waste stations	1	0.3	Buy
Laser and IPL platform	1	36.0	Lease
Radiofrequency or body contouring device	1	24.0	Defer
Longevity and Infusion			
Infusion recliner	4	3.6	Buy
Volumetric infusion pump	4	2.4	Buy
Vital signs monitor	2	1.8	Buy
Medication fridge	1	1.0	Buy
Body composition analyser	1	3.5	Buy
Trolleys and consumable stations	1	0.7	Buy
Hair Restoration, the FUE suite			
Motorised FUE punch and control unit	1	6.5	Partner
Implanter pens	4	2.0	Partner
Stereo microscope for graft preparation	2	3.6	Partner
Graft storage chiller	1	0.9	Partner
Instrument sets	1	1.2	Partner
Reclining procedure chair	1	2.5	Buy
Procedure light	1	1.8	Buy
Family Practice			
Examination couch	1	0.9	Buy
Vitals, otoscope, ophthalmoscope, spirometer	1	1.4	Buy
Vaccine fridge	1	1.0	Buy
Campus shared			
Defibrillator and resuscitation trolley	1	3.5	Buy
Emergency oxygen and suction	1	1.2	Buy
CSSD washer-disinfector and autoclave	1	6.5	Buy
Nurse stations, wheelchairs, patient trolley	1	2.0	Buy
Theatre suite			
AHU, HEPA filtration and ductwork	1	32.0	Defer
Theatre light, pendant and operating table	1	32.0	Defer
Anaesthetic machine and full monitoring	1	20.0	Defer

Item	Qty	NGN M	Route
Isolated power supply panel and UPS	1	11.0	Defer
Recovery, two monitored stations	1	10.0	Defer
Medbury Diagnostics and Medbury Pharmaceuticals			
Analysers, X-ray, ultrasound, echo, dispensary and cold chain		n/a	Those businesses
Route	NGN M	What it means	
Bought at launch	45.5	The cheap, essential and shared items. This is the only equipment line in the capital table	
Leased from month nine	36.0	One laser and IPL platform, at about NGN 1.2M a month with a buy-out option priced at signature. Not taken until injectable volume justifies it	
Deferred to phase two	129.0	The theatre suite and the body contouring device. Funded from trading, and the theatre only once a surgical anchor is signed	
Brought by the partner	14.2	The FUE instruments, which only the hair restoration operator will use. Nil if the partnership is agreed, NGN 14.2M if it is not	
Carried by the business that owns it	n/a	Laboratory and pharmacy, on Medbury Diagnostics' and Medbury Pharmaceuticals' own budgets	

The aesthetics unit opens injectables-led and buys no energy-based device. Injectables are the revenue anchor at about a third of the unit's turnover and they need a couch, a cold chain and an emergency kit. Lasers, IPL, radiofrequency and body contouring are the capital-heavy items, they are the ones distributors will lease rather than sell, and they are the ones whose demand is least evidenced in Abuja. **Taking the laser at month nine on a lease with a priced buy-out costs NGN 36M of capital nothing and answers the question with trading data rather than with an assumption.**

Every lease and placement in this schedule is to carry a **buy-out option priced at signature and exercisable from year two**. That keeps the lean structure a financing choice Medbury can reverse out of trading cash, rather than a permanent premium.

05 / THE PRINCIPLE

Why it is five prices and not one.

Element	Basis	Why that basis	When it is paid
Delivery	Fixed	A fixed fee, so CFA is paid for delivering the campus rather than for how much of Medbury's money passes through it	25% on signature, the balance monthly over the first 48 trading months
Capital efficiency	Share of the saving	CFA is paid to bring the campus in below the approved budget. The leaner it is delivered, the more CFA earns	Quarterly, out of the saving itself
Establishment	Per entity	Standing up a licensed clinical business is a fixed piece of work whatever that business later earns	25% at kickoff, the balance monthly over 36 months
Management, base	Revenue, with a floor	Funds and directs the operating team from the first trading month, when revenue is smallest and the work largest	Monthly
Management, incentive	Contribution above a hurdle	Nil unless the campus clears a 15% cash return on the capital deployed	Annual, in arrears
Equity	The operating company	The party that builds a business and runs it should own part of what it builds	On practical completion

The second line is the one worth pausing on. The usual development fee is a percentage of capital, which pays the developer more for spending more of the owner's money. On a mandate whose whole purpose is to open for the least possible cash, that is precisely the wrong incentive. **A fixed delivery fee plus a share of the saving reverses it: every naira CFA keeps off Medbury's balance sheet is a naira CFA is paid a fifth of.** It is the only fee line in this proposal where CFA's interest and Medbury's are identical rather than merely compatible.

06 / THE FEES

Each one, priced.

A. Delivery fee, and the capital efficiency share

	NGN M	What it covers
Delivery fee, fixed	90.0	Interior and clinical design, M&E coordination, procurement and negotiation of every equipment package including the leases and placements, contractor selection and management, the regulatory file, commissioning and handover
Mobilisation on signature, non-refundable	25.0	Credited against the fee
Balance, monthly over the first 48 trading months	65.0	NGN 1.35M a month
Capital efficiency share	47.4	20% of cash capital saved against the approved budget of NGN 450M, capped at NGN 50M. Paid quarterly out of the saving

The budget the efficiency share is measured against is set by an independent quantity surveyor and approved by Medbury before mobilisation, so it cannot be inflated to manufacture a saving. As a cross-check, NGN 450M against 711 sqm is NGN 633,000 per sqm, below the NGN 740,000 per sqm CFA published as the planning assumption for this asset class before this mandate was contemplated.

B. Entity establishment fee

NGN 22M per anchor clinical entity, plus NGN 6M per entity on the licence being granted. Distinct from the building, because a licensed clinical business is not a fitted-out room. It covers the structuring and shareholders' agreement, incorporation, facility registration and the FCT health licence, MDCN and NMCN credentialing, the clinical protocol and governance set, the recruitment brief and selection, the opening price list, and ninety days of operational hand-holding after first patient.

Entity	Establishment	On licence	Note
Medlyfe Aesthetics and Regenerative	22.0	6.0	Includes the NAFDAC and product import file
Medlyfe Longevity and Infusion	22.0	6.0	
Lyfe Place Hair Restoration	22.0	6.0	Includes the surgeon engagement terms
Alameda Conversion Clinic SPV	22.0	6.0	Includes the four amendments to the joint venture agreement
Four anchor entities	88.0	24.0	A fifth is priced the same way

NGN 5.5M per entity is payable at kickoff and the balance monthly over 36 trading months. The licence fee is payable on the licence being granted. Clinical and management recruitment is charged separately at 15% of first-year package, and only on placements Medbury accepts.

C. Management fee

Part	Basis	Timing	NGN M
Base	10% of campus-wide revenue, subject to a floor of NGN 7M a month in year one and NGN 10M a month thereafter	Monthly, from the first trading month	169.7
Incentive	15% of contribution to Medbury above a 20% cash-on-cash return on capital deployed	Annual, in arrears	44.1
Indexation	Both elements indexed at 12% a year, or CPI if higher	Annually on the anniversary	n/a
At stabilisation		12.6% of campus-wide revenue	213.8

The hurdle arithmetic at stabilisation: contribution to Medbury after the base fee is NGN 377M, the 20% cash-on-cash hurdle on NGN 417M of capital deployed is NGN 83M, and 15% of the NGN 294M above it is NGN 44M. **Two things about the base are deliberate.** The rate is set so that the management service earns a normal professional margin over the cost of fielding the team, rather than a percentage borrowed from a comparator table. And the floor is lower in year one than thereafter, because year one is when revenue is smallest and the work is largest, and a floor set at the mature level would take cash out of the campus in the month it can least afford it.

Campus-wide revenue is the single base for the whole mandate. Where CFA already holds a management agreement with an entity that trades on this campus, that entity's own fee is switched off for this site, so no naira of revenue carries two management fees.

D. Equity

Fifteen per cent of the campus operating company, issued on practical completion. The same proportion CFA holds in the Lyfe Place Aesthetics joint venture, and for the same reason: the party that designs a business, licenses it, staffs it and runs it should own part of what it builds. A fee buys CFA's attention for a year. An interest buys it for ten, and it is the difference between an operator who optimises the current quarter and one still building the fifth year of the business in the second.

07 / WHAT IT COSTS, AND WHAT IT RETURNS

Year by year.

NGN M	Setup	Yr 1	Yr 2	Yr 3	Yr 4
Campus-wide revenue	nil	976	1,663	1,697	1,697
Cash before the management fee	nil	110	546	567	567
Mobilisation and entity kickoff	47	nil	nil	nil	nil
Management, base	nil	98	166	170	170
Management, incentive	nil	nil	nil	44	44
Licence fees, on each licence granted	nil	24	nil	nil	nil
Deferred delivery and establishment	nil	nil	38	38	38
Total to CFA	47	122	204	252	252
Cash to Medbury, after the fee	nil	(12)	342	315	315
Medbury cumulative	nil	(12)	330	645	960
Medbury on the alternative, let by the session	nil	72	199	253	253

Medbury's capital comes back during year two. Cumulative cash after paying CFA in full reaches NGN 389M by the end of year two against NGN 284M put in before trading. On the session-let alternative the same capital returns in year three, and the capital itself is more than twice as large. **Year one is thin for both parties, and it should be: it is the year a campus is being built and four licensed businesses are being brought into existence.**

Only NGN 47M of CFA's setup fee is payable before the campus trades, against NGN 252M if it were charged conventionally. The balance is carried on the same terms CFA is asking Medbury to apply to its own equipment: paid out of the revenue it helps create, over 36 to 48 months, and in the case of the efficiency share, paid out of money that was budgeted and then not spent.

08 / THE TESTS

Whether the price is right.

Test one: what operators charge

Comparator	Rate	Note
Full management services organisation	8% to 20% of collections	The closest comparator, and this mandate is at the fuller end of that scope
Hospital and clinic management contracts	10% to 15% of revenue, plus incentive	Where an operator carries the clinical P&L rather than the building
Development management, healthcare fit-out	10% to 18% of capital	Design, procurement and delivery under one accountable party
Facility management only	4% to 6% of revenue	Buys a building run properly. Buys no clinical P&L and no businesses
This mandate	12.6% of campus-wide revenue	Mid-band, for a scope at the top of it

Rates are international comparators. There is no reliable Nigerian fee benchmark for this asset class, so they are offered as a fairness reference rather than as market evidence.

Test two: share of the value created

NGN M a year	Amount	Share
What the campus produces for Medbury, after the base fee	377	
CFA's margin on running it, after its own cost to serve	109	26%
Medbury keeps	333	74%
Medbury on the session-let alternative	253	On more than twice the capital

Test three: alignment

If this happens	This happens to CFA	Verdict
CFA opens the campus for less cash	The efficiency share pays CFA a fifth of the saving	Identical interests, not merely compatible
Revenue grows	The base fee grows with it	Aligned
Contribution grows	The incentive grows faster	Aligned
The campus clears less than a 15% cash return	CFA earns no incentive at all	Aligned
The campus is worth more in year ten	CFA's 15% is worth more	Aligned, and the reason for the equity
CFA delivers late	The deferred fee starts later and the incentive with it	Aligned. CFA is paid out of trading that has not begun

09 / THREE WAYS TO STRUCTURE IT

The same value, in different currency.

These are not three price points. They are three ways to settle the same value, and they differ in whether Medbury would rather pay in fee or in equity. A lower fee is bought with a larger interest, and the reverse.

Structure	Terms	Fee a year	Equity value	Assessment
1. Fee led	12% base, no incentive, no equity	204	nil	The simplest of the three. A known number, and Medbury keeps every naira of the upside above it and the whole company
2. Balanced	10% base, 15% incentive over a 20% hurdle, 15% of the operating company	214	218	Recommended. The structure priced throughout this document
3. Equity led	7% base, 10% incentive over a 20% hurdle, 25% of the operating company	150	363	Lowest cash cost by a distance. Medbury gives up a quarter of a company it expects to be worth about NGN 1.5Bn

Equity value is the stated share of an operating company earning about NGN 440M a year of contribution, valued at the 3.3 times multiple the Lyfe Place Aesthetics discounted cash flow implied. It is indicative and it is not a valuation.

Option two is the one CFA recommends, and option one is the one to take if simplicity matters more than alignment. The three cost Medbury within NGN 15M a year of each other at stabilisation, so this is genuinely a choice of currency rather than of price. What separates them is the bad year. Option one leaves CFA on a base fee and nothing else, which is precisely the year an operator is most tempted to reduce the team. Options two and three leave CFA holding an interest in the recovery, which is the year an owner most needs the operator to be building rather than defending a margin. **The equity is not a fee by another name. It is what keeps the operator in the room when the fee alone would not.**

10 / THE BOUNDARY

What the fee includes, and what it does not.

	Item
Included	The management team: campus director, operations, commercial, clinical governance, finance and billing oversight. CFA employed and directed
Included	Procurement and negotiation of every equipment package, including the lease, placement and consignment arrangements that keep them off the balance sheet
Included	Monthly management accounts by business unit, a quarterly board pack, and an annual operating plan and budget
Included	Tenant and entity relations, credentialing, the conversion clinic interface, and regulatory maintenance
Reimbursed	On-site staff: front office, nursing pool, records, security, cleaning. Engaged by CFA and reimbursed monthly
Reimbursed	Marketing spend, against a plan approved in the annual budget
Charged separately	Recruitment at 15% of first-year package, on accepted placements only
Excluded	Capital expenditure, lease and tariff payments, rent, utilities, insurance, clinical consumables, professional indemnity, and the clinicians' own fees
Excluded	Legal, tax, audit and the independent quantity surveyor. CFA instructs and manages, Medbury pays the adviser directly
Excluded	The CFA platforms. See the appendix

11 / TERM AND GOVERNANCE

How long, and how it ends.

	Term	Why
Term	Seven years from the first trading month, with a five year option	An operator cannot build a business on a horizon shorter than the asset, and the deferred fee is paid across the first four years
Delivery	Runs from mobilisation to commissioning, then converts to the management term	One continuous mandate, not two engagements
Exclusivity	CFA is the sole operator of the campus for the term	The mandate cannot be delivered alongside a second operator
Further sites	First right to manage any further Medbury facility on these terms	The platform being built here is what makes the second site cheap
Review	Annual operating plan and budget approved by Medbury before each year	CFA runs the campus. Medbury sets the plan
Reserved to Medbury	Capital expenditure, entity formation, pricing architecture, senior clinical appointments, any distribution	The owner's decisions stay the owner's
Termination for cause	90 days to cure a material breach, then immediate	Both ways
Termination for convenience	After year three, on 12 months' notice, with all deferred and accrued fees falling due and the equity retained	The deferred fee is CFA's money already earned. It cannot be extinguished by ending the mandate early
On exit	Orderly handover, records and patient data to Medbury	The clinical data is Medbury's. The platform and the systems are CFA's

12 / DECISIONS

What is needed to move.

	Decision	Why it is needed now
1	Confirm the medipark configuration	The fee, and the value it is measured against, both assume the businesses move in. A sessional letting building is a different mandate at a different price
2	Choose the structure	Option two, balanced, is recommended
3	Approve the benchmarked budget	The efficiency share is measured against it, so it must be set independently and agreed before mobilisation
4	Accept the lean financing principle	Leasing, placement and consignment cost NGN 51.5M a year of margin and save NGN 449M of capital. That trade has to be a decision, not an assumption
5	Confirm the rent actually paid	Sum and term. It sets the amortisation period and the capital base the hurdle runs on
6	Settle the aesthetics vehicle	Whether the Abuja unit sits inside the existing joint venture or is Medbury owned under a brand licence. It moves Medbury's annual take by about NGN 48M
7	Instruct the delivery	Design, the regulatory file and the solar and equipment lease negotiations are the long lead items. Every week of delay is a week of rent already paid and not trading

If they are settled this month

When	What
Weeks 1 to 2	Mandate signed, mobilisation paid, design instructed, quantity surveyor appointed
Weeks 3 to 10	Detailed design, regulatory file lodged, contractor selection, solar power purchase agreement and equipment leases negotiated
Weeks 6 to 20	Fit-out. Entity structuring and licensing run alongside, not after
Weeks 14 to 22	Recruitment, credentialing, protocols, pre-selling of memberships and programmes
Week 24	First floor opens and trades while the ground floor completes
Week 34	Conversion clinic opens. Theatre follows the surgical anchor

Opening the first floor at week 24 rather than waiting for the whole campus is worth about ten weeks of trading, and it is the single largest lever on how fast Medbury's capital comes back. It is a sequencing decision, not a spending decision, and it is available only because the businesses moving in upstairs do not depend on the theatre below them.

A / APPENDIX

The CFA platforms, available as an addendum.

Four CFA platforms would materially change how this campus is run, and none of them is included in the fees above. They are set out here so Medbury can decide whether to take them, separately and later, on their own terms. **Nothing in this mandate depends on them.** The campus can be delivered and run without any of the four, using whatever systems Medbury prefers, and the fees in section 06 assume exactly that.

Platform	What it is, and what it does here	What it replaces
HospitIOS	The operating system for a medical campus: booking and room status across every business, patient records, billing and collection, and unit-level reporting consolidated into one board pack	Six businesses each running their own booking, billing and records, reconciled by hand into a monthly report that arrives too late to act on
CadreHealth	The healthcare workforce platform at oncadre.com: sourcing consultants, resident clinicians, technicians and nurses, including a diaspora bench of Nigerian specialists abroad	Agency fees on every hire, a recruiter on payroll, and no access to the senior names the sessional panel depends on
Maarova	Assessment, selection and development for the management layer and the clinical leads, calibrated for healthcare rather than generic corporate use	Senior hires made on interview alone, in a campus where two or three wrong appointments would cost more than the platform ever would
The DFC panel	Access to the Doctors Foundation for Care network of diaspora physicians, for the specialist panel, visiting clinics and the conversion clinic's second opinions	Building a specialist panel from cold outreach, which is the slowest and least certain part of filling the sessional floor

Platform	How it would be priced	Indicative
HospitIOS	Campus licence, monthly, plus implementation and training	To be scoped
CadreHealth	Access licence, monthly, plus a placement fee per accepted hire	To be scoped
Maarova	Per assessment, on the current rate card. Coaching priced separately	To be scoped
DFC panel	Network access, monthly, plus an introduction fee per specialist onboarded	To be scoped

These are licensed, never contributed. CFA's platforms are its own assets and are made available to the ventures it operates under licence with operator economics. They are not part of the management fee, they are not consideration for equity, and they do not transfer on exit. That principle is applied consistently across every vehicle CFA operates with Medbury, and it is what keeps the platforms worth having.

Debo Odulana Consult for Africa / +234 913 813 8553 / hello@consultforafrica.com

Revenue, contribution and capital figures are taken from the Lyfe Place Abuja medipark model and carry that model's assumptions, of which aesthetics volume and sessional fill are the least evidenced. Lean capital figures assume lease, placement and consignment terms that are customary in this market but are not yet quoted, and each is to be confirmed at design stage. Comparator fee ranges are international and are offered as a fairness reference, not as Nigerian market evidence. Equity values are indicative and are not a valuation. Not a binding offer, and not legal or tax advice. FX reference USD/NGN 1,550.